

Name: Daniel S May | DOB: 10/11/1989 | MRN: 10105654130 | PCP: Joshua W Clark, NP | Legal Name:
Daniel S May

ED Provider Notes

Signed Jun 29, 2026

ED Provider Notes by Rahmatullah Rahmati, MD at 6/29/2026 2:18 PM



MASSACHUSETTS EYE AND EAR INFIRMARY EMERGENCY DEPARTMENT--OTOLARYNGOLOGY

Date: 6/29/2026

Patient: Daniel S May DOB: 10/11/1989 MEEI MRN: 8026121

PCP: Clark, Joshua W, NP

Assessment & Plan

1. Sensation of fullness in both ears

Persistent bilateral ear fullness with left side worse. Left ear tube visualized in place and dry, no evidence of infection.

- Discussed referral to otologist for further evaluation
- Refer to otologist for comprehensive evaluation

2. Otolgia of both ears

Reports bilateral ear pain, ongoing despite previous interventions. No acute infection observed on exam.

- Refer to otologist for further assessment

3. Mouth ulceration

Multiple oral ulcers present since 06/26/2026-06/27/2026. Lesions observed on tongue, hard palate, and cheek.

- Provided education on use of steroid paste for oral ulcers and expected healing time
- Prescribe steroid paste for topical application twice daily for 1 week

4. Chronic sinusitis, unspecified location

Chronic sinus congestion and mucus production for >1 year. Previous budesonide

and saline rinses not effective. Sinus CT from 08/2025 showed mild paranasal sinus disease.

- Discussed ongoing evaluation with new sinus specialist and importance of follow-up
- Reviewed prior sinus CT and MRI brain
- Continue evaluation with new sinus specialist (Dr. Lee)

5. Possible immune deficiency or autoimmune disorder

Recurrent oral ulcers and chronic ENT symptoms raise concern for immune dysfunction. No prior immune workup documented in this system.

- Discussed need for immune evaluation and possible autoimmune etiology
- Refer to allergy and immunology for immune workup

6. Gastrointestinal symptoms

Reports irregular stools and abdominal noises. No GI evaluation completed yet.

- Refer to gastroenterology for further evaluation

Attending: Rahmatullah Rahmati, MD

DAX Copilot AI assistant for documentation:

I obtained verbal consent from the patient or their proxy to record this visit for the purposes of producing a draft of the encounter documentation.

Chief Complaint

Patient presents with

- Sinus Pain

History of Present Illness

A 36-year-old male with chronic sinus and ear symptoms, history of septum surgery, and prior left ear tube placement seen for persistent sinus congestion, fullness, pain, and recent oral ulcers.

Persistent fullness in both ears and sinuses has been ongoing for over 1 year. Left ear symptoms are more pronounced, including clicking and a sensation of blockage. A tube was placed in the left ear by Dr. Krause, but no improvement was noted. Pain is reported in the upper jaw and ears, described as a pulling sensation. Chronic sinus and ear infections have resulted in constant mucus production and frequent nose blowing. No current use of nasal sprays or saline rinses. No allergy testing has been performed.

performed.

Two septum surgeries were performed approximately 12 years ago. A dentist rec identified a retained suture in the septum and an adjacent implant, but no abnormalities were found on x-ray. No history of sinus surgery beyond septoplast

Multiple oral ulcers have developed since the dental visit on 06/25/2026. No diffic swallowing is reported.

Vertigo was previously evaluated by Dr. Bery and has resolved. Azithromycin 300 weekly was prescribed for 3 months by infectious disease, without symptom relie

Gastrointestinal symptoms include irregular stools, nausea, and vomiting on 5 occasions. Thick mucus production and coughing are noted.

PAST MEDICAL HISTORY:

Past Medical History:

Diagnosis	Date
• Trauma to eye	

PAST SURGICAL HISTORY:

Past Surgical History:

Procedure	Laterality	Date
• SINUS SURGERY	Bilateral	

MEDICATIONS: @MEDENC@

ALLERGIES: No Known Allergies

CURRENT OUTPATIENT MEDICATIONS:

No outpatient medications have been marked as taking for the 6/29/26 encounter (Hospital Encounter).

ALLERGIES: No Known Allergies

EXAMINATION

Blood pressure (!) **132/92**, pulse 95, temperature 36.2 °C (97.1 °F), temperature source Temporal, resp. rate 18, height 188 cm (6' 2"), weight 81.6 kg (180 lb), SpO₂ 98%.

GENERAL APPEARANCE: well developed, well nourished patient in no acute distress.

VOICE: normal

BREATHING: normal, not labored, no stridor

FACE: no lesions. No tenderness to palpation

FACIAL STRENGTH: normal and symmetric

AD: pinna normal, no lesions or erythema. Canal clear. TM clear

AS: pinna normal, no lesions or erythema. Canal clear. Tube in place.

NOSE: Mucous membranes moist, no lesions, polyps, purulent discharge or bleeding

Septum: midline

Inferior Turbinates: enlarged bilaterally

ORAL CAVITY: superficial tender ulcers as pictured below.

OROPHARYNX: Soft palate normal, no lesions, normal elevation

Tonsils: 1+

Posterior pharyngeal wall: No cobblestoning

NECK: Trachea midline. No palpable lymphadenopathy.

No masses.

SALIVARY: Parotid glands normal, no tenderness, swelling or masses.

Submandibular glands normal size and shape, no tenderness.







Procedure: Nasal Endoscopy (CPT 31231)

Surgeon: RAHMATULLAH RAHMATI, MD

Indication: Chronic sinonasal symptoms unexplained by anterior rhinoscopy
Informed Consent: The details of the procedure including risks, benefits, and alternatives were discussed with the patient and verbal consent obtained to proceed.

Procedure: Topical spray consisting of 1% oxymetazoline and 1% lidocaine was sprayed into the nasal cavity bilaterally. A flexible nasopharyngoscope was then used to visualize each nasal cavity (septum, turbinates, middle and superior meatus, sphenoethmoidal recess), and nasopharynx. The oropharynx, larynx, hypopharynx were also visualized.

Findings: The nasal septum was deviated to the left. Inferior turbinates were hypertrophic. Left middle meatus: no edema, no purulence or polypoid. Left superior meatus: clear. Left sphenoethmoidal recess: clear. Right middle meatus: no edema, no purulence or polypoid. Right superior meatus: clear. Right sphenoethmoidal recess: clear. Nasopharynx and Eustachian tube orifices: No masses, lesions, or obstruction.

Oropharynx: clear

Larynx: clear

Hypopharynx: clear

Complications: None

Rahmati, Rahmatullah, MD

06/29/26 1525